# Summary of recommendation statements and practice points

## Chapter 1: Comprehensive care in patients with diabetes and CKD

### 1.1. Comprehensive diabetes and CKD management

**Practice Point 1.1.1: Patients with diabetes and chronic kidney disease (CKD) should be treated with a comprehensive strategy to reduce risks of kidney disease progression and cardiovascular disease (Figures 1 and 2).**

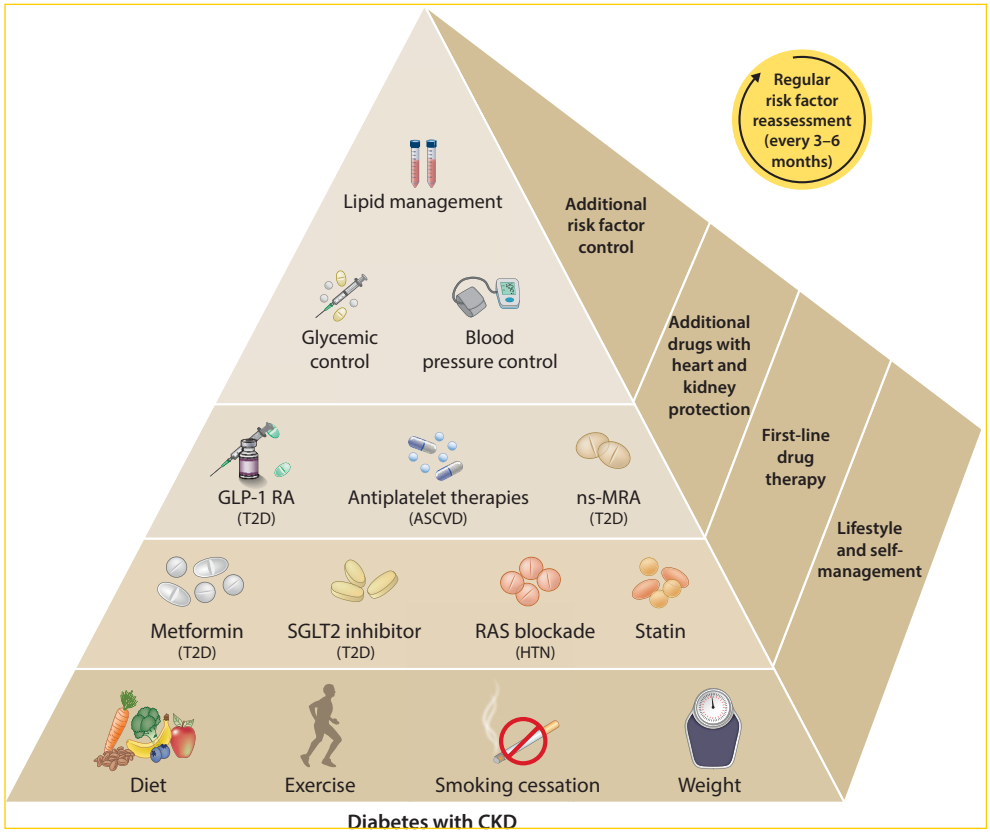


**Figure 1 | Kidney–heart risk factor management.** People with diabetes and chronic kidney disease (CKD) should be treated with a comprehensive approach to improve kidney and cardiovascular outcomes. This approach should include a foundation of lifestyle modification and self-management for all patients, upon which are layered first-line drug therapies according to clinical characteristics (in parentheses), additional drugs with proven kidney and heart protection as guided by assessments of residual risk, and additional interventions as needed to further control risk factors. Glycemic control is based on insulin for type 1 diabetes (T1D) and a combination of metformin and sodium–glucose cotransporter-2 inhibitors (SGLT2i) for type 2 diabetes (T2D). Metformin may be given when estimated glomerular filtration rate (eGFR) ≥30 ml/min per 1.73 $m^2$, and SGLT2i should be initiated when eGFR is ≥20 ml/min per 1.73 $m^2$ and continued as tolerated, until dialysis or transplantation is initiated. Renin–angiotensin system (RAS) inhibition is recommended for patients with albuminuria and hypertension (HTN). A statin is recommended for all patients with T1D or T2D and CKD. Glucagon-like peptide-1 receptor agonists (GLP-1 RA) are preferred glucose-lowering drugs for people T2D if SGLT2i and metformin are insufficient to meet glycemic targets or if they are unable to use SGLT2i or metformin. A nonsteroidal mineralocorticoid receptor antagonist (ns-MRA) can be added to first-line therapy for patients with T2D and high residual risks of kidney disease progression and cardiovascular events, as evidenced by persistent albuminuria (>30 mg/g [>3 mg/mmol]). Aspirin generally should be used lifelong for secondary prevention among those with established cardiovascular disease and may be considered for primary prevention among patients with high risk of atherosclerotic cardiovascular disease (ASCVD).